

Stereo.HCJDA-38

JUDGMENT SHEET

IN THE LAHORE HIGH COURT LAHORE

JUDICIAL DEPARTMENT

C.R. No.218702 of 2018

Shahzad Waheed

v.

Agha Khan University, etc.

J U D G M E N T

Date of Hearing	14.1.2021
Petitioner by	Ch. Israr Ahmad Bajwa, Advocate
Respondents by	Mr. Asad Javed, Advocate

Rasaal Hasan Syed, J. Petitioner in this civil revision has challenged the judgment and decree dated 28.2.2018 whereby the appeal of the respondent was allowed, decree of the learned Trial Court was set aside and, in result, suit of the petitioner was dismissed.

2. Facts from which the instant revision petition stems are that the petitioner instituted a suit for recovery of Rs.11,000,000/- as damages on account of expenses incurred and damages suffered due to laboratory report, which according to the petitioner was false and incorrect. It was claimed that as per doctor’s advice he gave his blood sample to carry out the test of P-ANCA and C-ANCA; the report issued reflected the P-ANCA and C-ANCA as “positive”; his doctor informed that the

diagnosis of the disease was serious which could cause death. Petitioner being mentally shocked travelled to United Kingdom for a second test which he got done from Cromwell Hospital, London and as per report, the P-ANCA and C-ANCA came as “negative”; the petitioner on his return again approached the respondent for the same test and after the requisite test and analysis, the report “corroborated” the report of Cromwell Hospital, London and that the petitioner suffered mental agony as well as financial losses because he had to spend considerable money on travelling to United Kingdom and boarding and lodging there and since he had to stay away and could not attend his business, his business bore the brunt in the form of losses as well. In this backdrop, the petitioner claimed damages.

3. The suit was resisted by the respondents who in their detailed written-statement explained each and every aspect of the matter and controverted the allegations levelled against them. Issues were framed. Petitioner appeared as P.W.1 and produced his driver as P.W.2. The respondents at their turn produced Farooq Ghani, Consultant Pathologist as D.W.1. The suit was partially decreed in the sum of Rs.1,000,000/- by the learned Civil Judge on account of travelling expenses

while the claim for mental torture and agony and financial losses was declined with the findings that the petitioner had failed to prove any criminal negligence on part of the respondents. Appeal filed by the respondents was accepted by the learned Addl. District Judge, Lahore vide judgment dated 28.2.2018. In result the decree of the learned Trial Court was set aside and suit of the petitioner was dismissed. In the instant revision petition, the judgment dated 28.2.2018 of the learned Appellate Court has been challenged.

4. Learned counsel for the petitioner submitted that the petitioner produced sufficient evidence to prove his plea of tests, stress, anxiety and mental torture which evidence was supported by laboratory report from the Cromwell Hospital, London, United Kingdom; the report of respondent No.1 was incorrect and false, due to which the petitioner suffered both financially as well as mentally and that the personal life of the petitioner was badly disturbed and that the learned Appellate Court did not consider the material evidence in its proper perspective and also that the impugned judgment was based on complete misreading and non-reading of material evidence adduced.

5. Contrariwise, learned counsel for the respondents vehemently opposed the stance of the other side and submitted that the learned Civil Judge decided issue No.2 against the petitioner and recorded findings to the effect that the petitioner had failed to prove criminal negligence, which findings were never challenged by the other side by way of cross-objections or at the time of hearing of the appeal and, even otherwise, there was no evidence to establish criminal negligence, in the absence whereof, the suit for damages was not maintainable and that it was a case in which material evidence was withheld and that the learned Appellate Court on indepth analysis of evidence rightly reversed the findings of the learned Trial Court and dismissed the suit and, therefore, no case for interference in revisional jurisdiction was made out.

6. Copies of the pleadings and evidence in the case as annexed with the revision petition have been duly examined to consider the points raised *pro* and *contra*. It is manifest therefrom that the case of the petitioner in the plaint was that on 24.2.2005, he gave a sample for the test of P-ANCA and C-ANCA at the Collection Point No.1 of respondent No.1 located at Main Boulevard, Defence Housing Authority, Lahore.

Analysis report was delivered on 05.3.2005. The petitioner was diagnosed to have P-ANCA value of 12.9 U/ml which was 6.9 U/ML more than to normal value and that C-ANCA was reported as 4.0 U/ml which was 2.0 U/ml above the normal range and that, as such, the petitioner was diagnosed P-ANCA and C-ANCA “positive” in the analysis report. In view of the “positive” report of P-ANCA and C-ANCA, his doctor, on consultation prescribed use of steroids and informed that the diagnosis of P-ANCA had severe implications on various organs of the body and death could result within a very short time. The petitioner claimed that having financial resources, he went to the United Kingdom for a second opinion, got his test of P-ANCA and C-ANCA which was carried out at the Cromwell Hospital, London and the report dated 24.3.2005 was given by Dr. Andrew Palmer, Consultant Nephrologist, whereby P-ANCA and C-ANCA were declared to be “negative” as opposed to the report dated 05.3.2005 of respondent No.1. It was added that the negative report gave a sense of life to the petitioner and that he had to incur huge financial expenses on account of travelling and boarding and also suffered tremendous

agony in terms of mental torture and his personal and family life were badly disturbed.

7. The petitioner claimed that he was a businessman who suffered losses on account of staying away from his business. It was added that on return to Pakistan, the petitioner once again gave a sample at the Collection Point of respondent No.1 and requested for another similar test and this time the result was “negative” which was delivered vide report No.L058802 dated 06.5.2005 and that this fact corroborated the result of Cromwell Hospital, London. The claim of petitioner was that due to the incorrect “positive” report petitioner and his family had undergone mental torture and he suffered financial losses and that all this happened due to the unprofessional handling of the test by respondent No.1.

8. In response respondent No.1 denied the stance of the petitioner and explained that the tests of P-ANCA and C-ANCA were specialized in nature and were intended to test the level of certain antibodies present in the blood and that such tests should only be prescribed when indicated by the “clinical presentation” of the patient and not in isolation and that the test could be prescribed for a condition known as Wegener *granulomatosis* which was extremely rare with a

classical clinical presentation and that any test results in isolation of such clinical presentation are to be first series of tests. It was further asserted that the laboratory report dated 05.3.2005 did not contain the diagnosis in respect of the petitioner and, in fact, it only reflected the “positive” test for both P-ANCA and C-ANCA which had to be associated with certain specified diseases and that it was intended merely to serve as a guide for interpretation of the clinical presentation. It was added that P-ANCA and C-ANCA results could not be used as sole and independent basis to diagnose the underlying disease and that the result of a test could only serve as possible indication of disease such as *Wegener granulomatosis* and could not be used to draw any independent conclusion as to diagnosis of a disease and that such diagnosis could only be made in conjunction with the clinical presentation of the patient and that such results must, therefore, be viewed in the context of clinical picture of the patient and the disease activity and in case a clinical presentation does not match the disease picture, such test results will bear little value and may give a false “positive” picture of *Wegener granulomatosis* and that the likelihood of existence of disease would be extremely low. It was asserted that,

accordingly, a “positive” report result in the presence of accompanying clinical presentation may be attributed to other causes, namely, the presence of another or different type of infective organism and/or seasonal variation the incidence of anti-proteinase 3 antibodies. It was thus stated that a “positive” result was, therefore, not diagnosis of a disease but should be treated as supplemental to clinical findings and diagnosis and not as a diagnosis in itself in the absence of any clinical findings. It was further stated that immunosuppressive therapy (i.e. a type of *steroid* therapy) should not be commenced on the basis of a “positive” P-ANCA result only and that any such treatment should be based on clinical observation coupled with a “positive” report. It was opined that notwithstanding the same, immunosuppressive/steroid therapy if commenced and administered to the petitioner after receipt of the first test result from respondent No.1, would reduce P-ANCA level and would cause C-ANCA level to decrease which, in consequence, could give “negative” reading in subsequent test. In this backdrop, the allegations of the petitioner as to unprofessionalism were denied.

9. In his evidence, the petitioner while appearing as P.W.1 deposed about the report from respondent No.1 and, thereafter, on consultation with doctor he claimed to have gone to the United Kingdom for a second test. As per P.W.1, the reports of Cromwell Hospital, London issued by Dr. Andrew Palmer, Consultant Nephrologist, declared P-ANCA and C-ANCA as “negative” which according to brought him sense of immense relief. Petitioner also deposed that on his return he had another P-ANCA and C-ANCA test done from respondent No.1 and this time the report came out as “negative” and that this “negative” second report “corroborated” the “negative” report of Cromwell Hospital, London.

10. It is evident from the plaint and also the statement of P.W.1 that he did not, either in the plaint or in his examination-in-chief, disclose as to the disease he was suffering from or the specific reason that occasioned his doctor advise P-ANCA and C-ANCA test. In his cross-examination, he deposed that he had pain in the “digestive system”. Further deposed that the doctor did not disclose the nature of the disease but advised him to get the test done. P.W.2, the driver of the petitioner, deposed that the petitioner was suffering from “kidney” disease. There was, therefore, inconsistency between the

statements of P.W.1 and P.W.2 as to the exact disease the petitioner was suffering from or the reason for which he was advised to get the P-ANCA and C-ANCA test conducted from respondent No.1. Petitioner in his cross-examination deposed that his doctor was Dr. Saima Waseem who had advised two tests and that she had subsequently *prescribed steroids* on receipt of the laboratory report. The petitioner did not produce either the prescription of the doctor or the doctor herself as his witness in the case.

11. As against his stance, the respondents in their written statement and also through evidence of the medical expert specifically maintained that the report of P-ANCA and C-ANCA tests itself would not be a diagnosis of any disease as clinical examination of the patient by the doctor, before and after the test was pivotal and that the test result could be “positive” for many reasons and further that without considering the clinical presentation, it would not be advisable to assume any particular disease nor steroids should be advised. In cross-examination as D.W.1, the witness deposed that he was working as Consultant Pathologist at the Department of Pathology, Agha Khan University Hospital and that he had been examining different

reports and that P-ANCA and C-ANCA itself was not a disease but was simply a blood test which is used for ascertaining different diseases. D.W.1 was specifically asked as to how such a person could survive if his test report reflects P-ANCA and C-ANCA as “positive”. The response of the witness was that P-ANCA and C-ANCA was simply a test and was not a disease and that such a test could be “positive” in different diseases. The witness further deposed that the “positive” report of P-ANCA and C-ANCA is not confirmative of any particular disease unless other tests such as biopsies were not conducted. The petitioner did not produce any expert evidence to contradict or rebut the statement of D.W.1 who was obviously a medical expert and his opinion carried substantial importance. It was, in these circumstances, that the petitioner was required to produce the doctor whom he consulted before or after the test and observations during clinical presentation should have been produced to corroborate the reason for getting those tests conducted. The clinical presentation before the doctor after the test reports was also essential to examine as to whether the doctor had ever informed the petitioner that there was a serious disease or that he could not survive so as to create panic for him and, if so,

what was the basis thereof. It was also not claimed by the petitioner that his doctor ever advised him to get *biopsy* before formulating any opinion about the nature of the disease.

12. So much so that the petitioner does not claim that after getting laboratory report Exh.P.1, he had ever undergone any *biopsy* for determination of the nature of disease nor did he claim that he ever had a second test done from some other reputable place in Pakistan like Shaukat Khanum Laboratories to get the same test repeated with a view to verify the report of respondent No.1. Instead he claimed to have flown out straightaway to the United Kingdom and got his second test done from the Cromwell Hospital, London. The report given by the doctor of the said hospital about P-ANCA and C-ANCA test was claimed to be “negative”. It is not the case of the petitioner that he ever presented the laboratory report Exh.P.1 given by respondent No.1 to the doctors in United Kingdom for getting their professional and expert view nor did he claim that they had ever commented adversely upon the report of respondent No.1.

13. To the contrary, the petitioner demolished his own stance by admitting that on return to Pakistan, he

again visited the Collection Point of respondent No.1, got another P-ANCA and C-ANCA test done and that the subsequent report about the P-ANCA and C-ANCA was “negative”. This shows that there could not be any criticism as to the professionalism or competency of the experts who had earlier given the test report Exh.P.1. In fact, the subsequent report corroborated the report of Cromwell Hospital, London which was positive evidence of the professionalism of the respondents and their competency in the matter.

14. The admission of the petitioner to the effect that he was advised to take *steroids* after the first test and that he went to London for a second test does not rule out the possibility of the petitioner having used the *steroids* and that is why the report from the Cromwell Hospital, London or the second report of respondent No.1 reflected P-ANCA and C-ANCA as “negative”. This aspect of the matter is corroborated by the expert view of D.W.1 who had not been contradicted when he deposed that due to use of *steroids* after his first test there was decrease in the level of P-ANCA and C-ANCA and this will be reflected in the second report.

15. Petitioner did neither produce any evidence in rebuttal nor any affirmative evidence to prove any

criminal negligence or lack of professionalism on part of respondent No.1. Even the findings of the learned Civil Judge were to the effect that the petitioner had failed to prove any criminal negligence on part of the respondents. These findings were not challenged by any cross-objections nor were they questioned at the time of hearing of the appeal, as evident from the impugned judgment of the learned Appellate Court. The learned Addl. District Judge, after indepth scrutiny of the entire evidence and taking care of the reasons of the Trial Court, rightly concluded that there was no iota of evidence to show any professional negligence or carelessness in preparation of test result Exh.P.1 and that the doctors in United Kingdom did not declare the report Exh.P.1 as false and the evidence on record reflected that the respondent had acted with extreme professionalism and that the second report on return of the petitioner to Pakistan, corroborated the report of the Cromwell Hospital, London, which reflected due professionalism and care in testing and preparation of reports by respondent No.1.

16. As to the visit to the United Kingdom, petitioner's own witness P.W.2 deposed that the petitioner had been visiting abroad, sometimes alone and at others with his

family and, in this view of the statement, it could not be assumed that the petitioner had any panic attack or went to the United Kingdom especially because of the earlier report Exh.P.1. Even otherwise, the claim of the petitioner as to financial losses or mental agony was declined by the learned Civil Judge, which was not challenged in appeal or cross-objections. As such, the same attained finality. In view of all these facts, the findings recorded by the learned Addl. District Judge to the effect that the petitioner was not entitled to any compensation and that the suit of the petitioner was liable to be dismissed, do not suffer from any misreading or non-reading of record.

17. As upshot of the above, there is no substance in the instant revision petition which is, accordingly, **dismissed**.

(Rasaal Hasan Syed)
Judge

*Announced in open Court on **04.2.2021**.*

Judge

Approved for reporting.

Judge